

MERCY GENERAL HOSPITAL
Department of Cardiology | Inpatient Services

DISCHARGE SUMMARY

Document No: DS-2024-00447	Admission Date: March 14, 2024	Discharge Date: March 19, 2024
Attending Physician: Dr. Anita Patel, MD, FACC		

Patient Name: Margaret Louise Williams	Date of Birth: August 3, 1949 (Age: 74)
MRN: MGH-774-2291	Sex: Female
Insurance: Medicare Part A & B	Discharge Disposition: Senior Living Facility — Sunrise Gardens

PRIMARY DIAGNOSIS

Acute decompensated heart failure (HFrEF), EF 35%
Secondary: Type 2 diabetes mellitus, Hypertension, CKD Stage 3

REASON FOR ADMISSION

Patient presented to the ED on 3/14 with worsening dyspnea on exertion, bilateral lower extremity pitting edema (+3), and orthopnea x3 days. BNP on admission: 1,840 pg/mL. CXR demonstrated pulmonary vascular congestion and bilateral pleural effusions. Patient was admitted for IV diuresis and cardiac monitoring.

HOSPITAL COURSE

Patient received IV furosemide 80mg BID x3 days with good diuretic response. Total fluid removed: approximately 4.2L over admission. Repeat BNP at discharge: 620 pg/mL. Metoprolol succinate dose was uptitrated from 25mg to 50mg daily. Lisinopril was continued. Blood glucose management was optimized with sliding scale insulin. Patient was euvolemic at time of discharge with stable oxygen saturation on room air.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Notes
Furosemide (NEW)	40mg	PO	Once daily (AM)	CHANGED from 20mg — monitor weight daily
Metoprolol Succinate (CHANGED)	50mg	PO	Once daily	UPTITRATED from 25mg
Lisinopril	10mg	PO	Once daily	Hold if SBP <90
Spironolactone (NEW)	25mg	PO	Once daily	Monitor K+ in 1 week
Atorvastatin	40mg	PO	Nightly	Continue unchanged
Metformin	500mg	PO	Twice daily	Hold if contrast needed
Aspirin	81mg	PO	Once daily	Continue unchanged

DISCHARGE INSTRUCTIONS

- Weigh yourself every morning before eating. Call your doctor if you gain more than 2 lbs in 1 day or 5 lbs in 1 week.
- Follow a low-sodium diet (<2g sodium per day). Avoid canned foods, processed meats, and adding salt.
- Fluid restriction: 1.5 liters per day.

4. Take furosemide in the morning to avoid nighttime urination.

5. Do not stop any heart medications without consulting your physician.

[!] Follow-up appointment with cardiologist: NOT SCHEDULED — facility to arrange within 7 days.

[!] Potassium level recheck: Required in 5-7 days — no lab order placed at discharge.

ALLERGIES

Penicillin — anaphylaxis

Sulfa drugs — rash (NOTE: Furosemide is a sulfonamide derivative — prescriber aware, benefit outweighs risk)

PENDING RESULTS AT DISCHARGE

Urine culture (collected 3/18) — results pending, facility to follow up with MGH lab at 617-555-0192